

Resupply of Hormonal Contraception

Assess patient needs to determine whether resupply is safe and appropriate → Review MHR, where possible/available → Provide **one original pack (up to 4 months' supply)** depending on product). → Reminder that reviews by GP or authorised prescribing health practitioner is required to **occur at least every 2 years**

Questions	Answer here	For pharmacist
Age: Is the patient aged 16-50 years?	Age years	[Triggers: Age <16 or >50 → Refer to GP]
Consent: Does the patient consent?*	Yes / No	[Triggers: No → Refer to GP]
*The patient must be physically present in the pharmacy		
Medicines List: Is the patient using a hormonal contraceptive included in Medicines List?	Use Yes / No	[Triggers: No → Refer to GP]
Was this hormonal contraceptive initiated by a GP (or other authorised prescribing health practitioner)?	Yes / No	[Triggers: No → Refer to GP]
Has the patient seen their GP (or other authorised prescribing health practitioner) within the last 2 years to review and confirm the appropriateness of continuing their current hormonal contraceptive?	Last saw GP on / /	[Triggers: more than 2 years ago → Refer to GP]
Has the patient been stabilised on their current hormonal contraceptive?	No / Yes ○ Patients aged 16-17 years must be using the same hormonal contraceptive for at least 12 months ○ Patients aged ≥ 18 years must be using same hormonal contraceptive for at least 6 months	[Triggers: No → Refer to GP]
Has the patient been continuously using the hormonal contraceptive: *** Consider the need for emergency contraception and/or testing to exclude pregnancy (note: pregnancy tests can give a false negative result if unprotected sex was less than 21 days earlier).	No: ○ No (had a break of ≤ 2-4 weeks) ○ No (had a break of > 4 weeks) Yes: • Without a break, OR • Had a break of less than 2 weeks during the preceding 30 days?	[Triggers: NO] (If using the OCP or vaginal ring and the patient has had a break of 2-4 weeks in the preceding 30 days, → the patient may be supplied ONE month of treatment AND must be referred to their GP for review.) (If using the OCP or vaginal ring and the patient has had a break of more than 4 weeks in the preceding 30 days OR if the patient is using the depot medroxyprogesterone acetate injection and is late (i.e. 14 weeks since their last injection) . → The patient is not eligible for resupply of their hormonal contraceptive under this protocol and must be referred to their GP for review.)
Conduct clinical review ○ Current medications ○ Medication allergies and/or adverse effects , including with hormonal contraceptive ○ Changes in bleeding patterns ○ Changes in health that would be a contraindication to ongoing use, e.g. angina, heart attack, stroke/TIA, breast cancer, liver disease, DVT/PE, migraine with aura, new headaches, increased VTE risk (e.g. recent major surgery) ○ Smoking/vaping status ○ Where clinically relevant, measure or review changes in BP. BP should be measured at least annually. Measurements taken within the last 12 months are acceptable where the patient's health status has not changed. ○ Where clinically relevant, review BMI (measure height and weight as necessary). BMI should be measured at least annually. Weight measurements taken within the last 12 months are acceptable where the patient's health status has not changed. ○ Confirm hormonal contraceptive is still appropriate and meeting patient's needs, considering: • Contraindications • Medication interactions • Pregnancy and lactation • Potential pregnancy		
EXCLUSION: Do any of the following exclusion criteria apply?	No / Yes ○ Requested hormonal contraceptive is now contraindicated (UKMEC 4) ○ requires specialist consideration (UKMEC3) ○ Not appropriate. Considerations may include hypertension, BMI > 35, current breast cancer, increased VTE risk that requires medical review, etc. ○ Unexplained and un-investigated symptoms including vaginal bleeding, severe or painful menstrual bleeding, irregular periods, amenorrhoea, or pain/discomfort with sexual intercourse ○ Signs and symptoms of PCOS that have not been assessed by an appropriate healthcare provider ○ Potentially pregnant	[Triggers: Yes → Immediate referral to GP]
TREAT & REFER: Do any of the following 'treat and refer' criteria apply?	No / Yes ○ STI screening is indicated ○ CST (Cervical screening test) is indicated ○ Possibility of reproductive coercion, sexual abuse or sexual violence	[Triggers: Yes → Treat and concurrently refer to GP]

Medicines list

Combined oral contraceptive pills (COCP)

Monophasic

Low estrogen dose	Estradiol 1.5mg	Nomegestrol 2.5mg	(24+4) Zoely  (this COC had higher incidences of missed withdrawal bleeds (frequency increased over time), breakthrough bleeding/spotting, acne and weight gain)
	EE 20mcg	Drospirenone 3mg	(24+4) Yaz, Yana, Rosie, Brooke, Bella  (the shortened hormone-free interval may increase contraceptive effectiveness by further suppressing ovarian function)
	EE 20mcg	Levonorgestrel 100mcg	(21+7) Femme-Tab ED 20/100, Micronelle 20 ED, Microgynon 20 ED, Loette   
Standard estrogen dose	Estetrol 14.2 mg	Drospirenone 3 mg	(24+4) Nextstellis 
	EE 30mcg	Drospirenone 3 mg	(21+7) Yasmin, Yelena, Rosalee, Brooklynn, Isabelle, Petibelle  
	EE 30mcg	Levonorgestrel 150 mcg	(21+7) Femme-Tab ED 30/150, Micronelle 30 ED, Microgynon 30 ED, Monofeme, Levlen, Lenest 30, Leveth 150/30, Evelyn 150/30, Eleanor 150/30,          **(84+7; 7 of EE10mcg) Seasonique (Seasonique® does not have a hormone-free interval, unlike other COCs) 
	EE 30mcg	Desogestrel 150 mcg	(21+7) Marvelon, Madeline 
	EE 30mcg	Dienogest 2mg	(21+7) Valette
	EE 30mcg	Gestodene 75 mcg	(21+7) Minulet 
	EE 35mcg	Cyproterone 2 mg (associated with an increased risk of VTE compared with other COCs)	(21+7) Estelle-35, Diane-35, Brenda-35, Jene-35, Juliet-35 (Indication: Androgenisation and contraception in these women)      (Contraception in absence of androgenic symptoms—use alternative COC.) (meningioma) (Pregnancy Theoretically could cause feminisation of male fetus)
	EE 35mcg	Norethisterone 0.5 mg	(21+7) Norimin, Brevinor  
	EE 35mcg	Norethisterone 1 mg	(21+7) Norimin-1, Brevinor-1, Pirmella   

Multiphasic

Tri-phasic	EE+Levonorgestrel (mcg) 30/50 (#6)→ 40/75 (#5)→30/125 (#10)	(21+7) Triquilar ED, Trifeme, Triphasil, Logynon ED    
4-phasic	Estradiol valerate + Dienogest (mg) 3/nil (#2)→ 2/2 (#5)→ 2/3 (#17)→ 1/nil (#2)	(26+2) Qlaira (→ compared to placebo, Qlaira® is effective for heavy menstrual bleeding)  (Women taking the pill can sometimes miss a period. This occurs more often with Qlaira®)

Progestogen only pills (POP) oral contraception

Norethisterone 350mcg	Levonorgestrel 30mcg	Drospirenone 4mg
(28) Noriday 28-Day 	(28) Microlut 	(24+4) Slinda, Lucinda 

Combined hormonal contraceptive vaginal ring

Vaginal ring	EE 2.7 mg (15 mcg/24 hours)	Etonogestrel 11.7 mg (120 mcg/24 hours)	NuvaRing 
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Depot medroxyprogesterone injection

Medroxyprogesterone 150mg/mL Inj.	Depo-Provera, Depo-Ralovera 
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The following hormonal contraceptives have been excluded from the Program:

- COCPs with a high estrogen dose (50mcg of EE or equivalent) - not routinely recommended for contraception due to unacceptable risk of VTE (Microgynon 50)
- Etonogestrel subdermal contraceptive implant (Implanon NXT)
- Levonorgestrel-releasing intrauterine contraceptive devices -IDU (Kyleena, Mirena)

***A pharmacist with suitable premises and competency in administration of **deep IM injection** may administer depot medroxyprogesterone.

Contraindication

COCs & Vaginal Rings

Table 3: Contraindications and precautions for combined hormonal contraception:¹⁰

UKMEC Criteria	Conditions
UKMEC 4 – Contraindicated	- current breast cancer - the first 6 weeks postpartum if breastfeeding (but safe to use after this timeframe) - the first 3 weeks postpartum in individuals with additional risk factors for venous thromboembolism (VTE) migraine with aura within the last 5 years - current or history of ischaemic heart disease, stroke or transient ischaemic attack aged 35 years or older and smoking either 15 or more cigarettes per day (or any amount of nicotine vaping according to Australian consensus guidelines) hypertension (systolic blood pressure 160 mmHg or higher, diastolic blood pressure 100 mmHg or higher) - complicated valvular or congenital heart disease - current or history of VTE - positive antiphospholipid antibodies - known thrombogenic mutation major surgery with prolonged immobilisation - severe (decompensated) cirrhosis - hepatocellular adenoma or malignant liver tumour.
UKMEC 3 – requires contraceptive specialist consultation	- body mass index (BMI) more than 35 kg/m² diabetes with nephropathy, retinopathy, neuropathy or other vascular disease - history of migraine with aura, but no episodes within the last 5 years - multiple risk factors for cardiovascular disease gall bladder disease (medically treated or current) - a first-degree relative with a VTE (provoked or unprovoked) under the age of 45 years - between 0 to 3 weeks postpartum and do not have additional VTE risk factors (and are not breastfeeding) [Note 1] - between 3 to 6 weeks postpartum and do have additional VTE risk factors (and are not breastfeeding [Note 2]). Note 1: Use of combined hormonal contraception before 6 weeks is contraindicated (UKMEC 4), if breastfeeding, to limit possible (unproven) impact on neonatal growth. Note 2: Use of combined hormonal contraception before 6 weeks is contraindicated (UKMEC 4), if breastfeeding, to limit possible (unproven) impact on neonatal growth.

POPs

Table 6: Contraindications and precautions for progestogen-only contraceptive pills:¹⁰

UKMEC Criteria	Conditions
UKMEC 4 – Contraindicated	Current breast cancer
UKMEC 3 – requires contraceptive specialist consultation	- unexplained vaginal bleeding (suspicious for a serious condition) before investigation for the cause past breast cancer - severe (decompensated) cirrhosis - hepatocellular adenoma or malignant liver tumour ischaemic heart disease, stroke or transient ischaemic attack (TIA) that develops during use.

DEPO-MPAs

Table 8: Contraindications and precautions for depot medroxyprogesterone contraception^{9,10}

UKMEC Criteria	Conditions
UKMEC 4 – Contraindicated	Current breast cancer
UKMEC 3 – requires contraceptive specialist consultation	- unexplained vaginal bleeding (suspicious for a serious condition) before investigation for the cause past breast cancer - severe (decompensated) cirrhosis - hepatocellular adenoma or malignant liver tumour - multiple risk factors for cardiovascular disease (eg older age, smoking, diabetes, hypertension, obesity) hypertension, with vascular disease - history of ischaemic heart disease, stroke or transient ischaemic attack.

While not included in the UKMEC categorisation, some caution is required if considering depot medroxyprogesterone in individuals with known low **bone mineral density (BMD)**, or risk factors for low BMD; this includes those younger than 18 years or older than 45 years.

Unlike other progestogen-only methods of contraception, depot medroxyprogesterone use is associated with a theoretical but unproven increase in the risk of **cardiovascular disease (likely due to its adverse effect on lipids)**. Use of depot medroxyprogesterone in individuals with multiple risk factors for, or a history of, cardiovascular disease requires specialist advice.⁹